

# Using This Course With Others

**Leaders' Guidance for *When Your Mind Won't Rest*, a Christian course for overthinking, worry, fear, and renewing your mind.**

*Reading this guidance does not qualify anyone as a counsellor or mental-health professional. Churches and group leaders must follow applicable safeguarding law and their own organisational procedures. Nothing in this document is legal advice.*

This is the course's leaders' section in one document, for pastors, counsellors and small-group leaders. It is the same guidance that appears on the site, with the printable Group Agreement and Leader's Checklist at the end.

**Two worksheets stay out of the group entirely:** "If This Happened, What Would It Mean to Me?" (Lesson 4) and The Faithful Decision Sheet (Lesson 18). A member may mention that they used them. Nobody should be asked what is on them.

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## **What This Course Is, and What It Is Not**

This is a pastoral course, built from a pastoral book. It teaches Christians to understand a restless mind, to recognise the patterns overthinking falls into, to test those patterns against evidence and Scripture, and to build a life that supports peace. Its tools are pastoral tools. Several of them resemble techniques found in cognitive and behavioural therapies, and that resemblance is not accidental, because good pastoral care and good therapy both attend to what a person believes and does. But the resemblance does not make the course a therapy, and it does not make you a therapist by using it.

The course is not a treatment for generalised anxiety disorder, panic disorder, obsessive-compulsive disorder, post-traumatic stress, depression, or any other clinical condition. It says so in every chapter, and it will say so in your group. It is not a screening instrument. It is not designed for self-directed exposure work, and it must not be adapted into it. And it is written for adults. This edition should not ordinarily be used as a group course for minors. Any adapted use with teenagers requires appropriately trained youth leaders, the organisation's safeguarding approval, age-appropriate materials, and the consent arrangements required by local law and policy; the exercises on fear, memory and decisions are not designed for minors, and parental knowledge alone is not a sufficient safeguard.

What the course can do, in your hands, is give a common language, a set of practices that a person can use between conversations, and a structure for six weeks of honest reflection in Christian community. That is worth a great deal. It is also the limit.

## Who Should Lead

You do not need clinical training to lead a group through this course, and most of the people who will use it with others will be pastors and mature lay leaders. But you do need several things.

You need to have read the whole course, including every help note, before the first session, and to be able to say without hesitation what the course is not for.

You need to know your own limits and be at peace with them. A leader who feels they must be able to handle whatever arises is a leader who will handle some things badly. The single most important skill in this section is knowing when to say, “This is beyond what I can help with, and I want to help you find someone who can.”

Before leading, at least one leader should have completed current basic safeguarding training and recognised suicide-awareness or suicide-first-aid training appropriate to the country. Reading this guidance is not a substitute for that training; it prepares you to recognise danger and activate professional help, which is what a lay leader needs to be able to do.

You need to know the referral routes in your area before you need them: how a person sees a doctor, how mental-health services are accessed, what specialist services exist for domestic abuse, for bereavement, for eating disorders, for substance use, and what the emergency numbers are. Write them down. Keep them with you in every session. Services and numbers change; check them at the start of each course.

You need a second person. A small group on this material should have two leaders, or one leader and a designated person who is not in the group whom the leader can consult. One-to-one work should sit within the accountability structures your church uses for pastoral care generally. Nobody should do this work alone.

And you need to be honest with yourself about your own restlessness. Many people are drawn to lead on this subject because they know it from the inside. That can be a gift. It can also mean that a group member’s story lands on your own unhealed material. Have someone you can talk to, and use them.

## Confidentiality and Its Limits

People will disclose things in a group like this that they have told no one. That is part of why the group is valuable, and it creates obligations.

At the first session, before any sharing, state the confidentiality agreement plainly. Explain that every member is expected to protect the privacy of others: what is said in the group stays in the group, and members do not discuss one another's disclosures outside it, even with spouses. Be honest that leaders can set and reinforce this agreement but cannot guarantee the conduct of every participant outside the group, and that members should share accordingly.

Then explain the circumstances in which information may need to be shared, because confidentiality has limits and it is dishonest to imply otherwise: safeguarding concerns, serious risk of harm to the person or to others, applicable legal duties, and necessary consultation with a safeguarding lead or professional supervisor. Say that in those circumstances you will share only what is necessary and only with those who need to know, and that you will, wherever possible, tell the person first what you intend to do. Say that you will not promise to keep secret anything that would leave someone in danger.

Do not make promises about what the police, social services or health services will do with information. You do not control that, and reassurances about it may be wrong.

Keep only the records that applicable law, safeguarding procedures and professional standards require. Record factual information, decisions and actions rather than speculation. Store records only in the organisation's approved secure system, and do not keep disclosures in an informal personal notebook; a leader's private notebook of group members' disclosures is a safeguarding problem waiting to happen.

## Safeguarding

If anyone in the group discloses that a child or a young person is being harmed, or that a vulnerable adult is being abused or neglected, or if anything said in the group gives you reasonable cause to believe this, you must follow your church's safeguarding procedure and, where the law requires it, report to the statutory authorities. This is not a matter for pastoral discretion, and it is not something to pray about first and decide later. If your church has a safeguarding lead, contact them the same day. If it does not, that is the first thing to put right.

The course contains material on repairing past wrongs, on carrying other people, and on family responsibility. In a group, that material may bring disclosures of historic abuse, of current abuse, or of a member's own harm to others. You are not equipped to investigate any of these, and you should not try. Your role is to listen, to take the person seriously, to make sure they are safe now, to follow the procedure, and to help them reach the people whose job it is.

## Suicide and Self-Harm

Some people who read this course are in more danger than the course's tone suggests. A person who cannot rest their mind may also, on some nights, not want to be here. You may be the first person they have told.

If someone discloses thoughts of suicide or self-harm, in the group or privately, stay with it. Do not change the subject, do not minimise, do not respond with a Bible verse in place of a conversation, and do not treat it as a spiritual failure. Ask directly and calmly whether they are thinking about ending their life. Asking does not plant the idea; it tells the person you can bear to hear the answer. Ask only enough direct questions to understand whether urgent danger may be present; do not invite unnecessary descriptions or explore methods in detail, and pass what you learn promptly to the emergency or crisis professional assisting you. Ask whether they believe they can remain safe, but do not rely on reassurance alone.

If they have current intent, a plan, access to means, a recent attempt, cannot remain safe, or you remain uncertain, treat it as an emergency: contact the emergency services or an urgent crisis service, stay with the person or arrange for someone to, and do not leave them alone while help is on its way. Do not promise confidentiality; you cannot keep such a promise and should not offer it.

If they are not in immediate danger, help them reach a doctor or a mental-health service promptly, ask whether there is someone they trust who can know, and follow up within a day. Write down what was said and what you did, and tell your safeguarding lead or the person you are accountable to.

Do not create a detailed clinical safety plan independently. Contact an emergency or crisis professional and follow their instructions about immediate safety and about reducing access to dangerous means. Do not personally handle weapons or place yourself in danger.

After the immediate response, address the group without inviting discussion of the person's private circumstances. Wherever possible, consult the person about what may be said. Share only the minimum necessary, and follow safeguarding and organisational policy.

Telling a person in crisis that suicide is unforgivable can deepen shame, fear and hopelessness. It must not form part of any crisis response, and it must not be spoken by anyone leading on this course. Offer compassionate presence, and obtain immediate help. Grace is the message. Help is the action.

## Domestic Abuse and Coercive Control

Lessons 8, 9, 12 and 18 in particular may surface disclosures of a relationship in which a member is being harmed, controlled, frightened or isolated. The course has been careful to say, each time, that safety comes before disclosure and that safety planning belongs with a specialist service. You must hold the same line.

Do not attempt mediation or couple work where abuse is disclosed. Do not advise reconciliation, submission, more prayer, or “giving it time.” Do not contact the other partner. Do not counsel the person to confront, to leave immediately, or to stay; leaving is often the most dangerous moment, and the timing is a matter for the person and a specialist. Do not disclose what you have been told to family members, general church leaders or anyone who may increase the danger; the course says, correctly, that safety advice may need to come before disclosure to family or church leaders, and that applies to you. For a competent adult, seek consent before sharing wherever possible. Exceptions may apply where children or vulnerable adults are at risk, where serious imminent danger exists, or where law and safeguarding policy require action; in those cases consult the safeguarding lead or the specialist service, and disclose only what is necessary.

Do help the person reach a domestic-abuse service, offer to sit with them while they make the call, and be one place in their week where they are believed. Before you contact them afterwards, ask whether it is safe to call, text, leave a message or send material; a monitored phone or email account can create additional danger. If children are involved, safeguarding procedures apply.

And be careful with the course’s own material here. Worksheets on “other possible explanations,” on “what is mine to carry,” on “release,” and on “faithful, wise or perfect,” can be turned by a person in an abusive relationship into reasons to excuse, to endure, or to blame themselves. The course has tried to guard against this in each place. In a group, you are the second guard. If a member’s use of a worksheet consistently produces a more charitable reading of someone who is frightening them, notice it, and speak with them privately.

## **Responding to a Disclosure**

Whatever the disclosure, the shape of a good response is the same, and it is worth having it in front of you.

Listen without interrogating. Thank the person for telling you. Do not promise secrecy. Establish whether immediate safety is involved. Explain what you must do next, and why. Record facts and actions according to policy, not impressions. Consult or refer promptly. Follow up, without assuming responsibility that belongs to professionals.

## Referral: When to Send Someone On

The front-matter list in “When This Course Is Not Enough” is your referral threshold, and you should keep it immediately accessible in every session; during a crisis, a written protocol is safer than memory. It says: thoughts of suicide or self-harm; panic attacks; an inability to function at work, at home or in relationships; symptoms following trauma; abuse or danger; severe or persistent anxiety or low mood that has not responded to ordinary support; obsessive- compulsive behaviours, including religious scrupulosity. Any of these means professional help, and the course is at most a companion to it.

Two more thresholds belong to the group setting.

First, the practices themselves. If a member reports that an exercise has become compulsory, that they are redoing worksheets to feel certain, that missing a practice produces disproportionate distress, or that they cannot stop, the course’s tools have met something they are not designed for. Stop the member’s use of the relevant practice, and help them reach a professional who understands obsessive- compulsive presentations. Do not respond by refining the worksheet.

Second, decline. If a member is visibly worse over the weeks, missing sessions, withdrawing, or increasingly hopeless, do not assume the course will get there eventually. Speak with them privately and help them find more.

When you refer, do it well. Say what you have noticed, without diagnosing. Say that this is beyond what the group or you can offer, and that this is not a judgement on their faith. Help them with the practical steps; the hardest part of getting help is often the first phone call. Ask whether they would like you to remain involved, and, if they consent, whether their clinician would find it useful for you to be in contact; pastoral and professional care can sometimes be coordinated, but only with the person’s consent and never over their head. Then keep caring for them. Referral is not handing someone off.

Never advise anyone to start, stop or change medication. Never describe medication as a failure of faith or as something to be prayed away. If a member asks your view, say that it is a matter for them and their prescriber, and that you will support whatever they decide together.

## **The Worksheets Are Not Diagnosis, and Sharing Is Not Compulsory**

Two errors are so easy to make in a group that they deserve their own heading.

The first is treating a worksheet as though it revealed a condition. It does not. A person who completes the Overthinking Reflection and finds that their mind goes most often to the future does not “have” catastrophising, and a leader who says so has made a diagnosis they are not qualified to make. The worksheets describe patterns in ordinary language so that a person can recognise their own; they are not instruments, they are not scored, and the course removed numeric scoring on purpose. Do not put it back. Do not invent categories of member. Do not compare members’ pages.

The second is compulsory disclosure. Every reflection and exercise in the course is written to be done privately, and several of them touch memory, fear, past wrongs and current relationships. No member should ever be asked to share what they wrote. Offer a private alternative for every sharing moment: “You may share something from this page, or you may share what the chapter made you think about, or you may pass.” Say it every week, so that passing is normal. Watch for the member who shares because the room seems to expect it, and protect them. Some of the worksheets, the Lesson 4 exercise (“If This Happened, What Would It Mean to Me?”) and the Faithful Decision Sheet especially, should generally not be shared in a group at all; use them in the week, and discuss the teaching rather than the page.

If a member does disclose something painful, thank them, do not let the group rush to fix or advise, and check in with them privately afterwards. If the disclosure is one that triggers the safeguarding, suicide or domestic-abuse guidance above, follow it.

## Running the Six Weeks

The course was written to be walked through in six weeks, and the Start Here page “How This Course Works” suggests the shape. Six weeks for twenty-one lessons is a guided overview, not a lesson-by-lesson study: leaders select the central chapters for each week, and participants should not attempt every worksheet. One carefully chosen practice each week is enough, and the course is not completed by covering every question. If your group would be better served by more time, run it over eight weeks by splitting Weeks One and Two below. Here is one way to run the six-week version. Adapt freely; the structure matters less than the boundaries.

**Before the course.** Members read "Welcome," "How This Course Works" and "When This Course Is Not Enough." Make "If You Are Not Sure You Belong to Christ" available to anyone who wants it, without treating it as required preparation. Leaders read everything, including this section, and have the referral list ready.

**Week One: Understanding the Restless Mind (Lessons 1-5).** Introduce the course and read the **Group Agreement** aloud: the confidentiality agreement, its limits, the private alternative for sharing, and the fact that nobody in the room will be told their anxiety is weak faith. Discuss thinking versus circling, the shame Christians attach to anxiety, and the body. Members choose one of the five exercises for the week.

**Week Two: Recognise the Pattern, part one (Lessons 6–9).** Predicting disaster, replaying the past, rehearsing conversations, reading minds. Teach the evidence question of Lesson 6 well; it recurs throughout. Note the safety material in Lessons 8 and 9.

**Week Three: Recognise the Pattern, part two (Lessons 10–12).** Certainty-seeking, perfectionism, carrying everyone. Lesson 10 will produce the most discussion, and the most theological disagreement about guidance; keep it charitable and keep the principles on signs and impressions in view. Lesson 12 will surface carers; care for them.

**Week Four: Renew Your Mind (Lessons 13–15).** Take every thought captive, renewal versus positive thinking, what is true. Guard against turning Lesson 13 into thought-policing; the course is explicit that it is not.

**Week Five: Prayer and Tomorrow (Lessons 16–18).** The Six-Movement prayer, giving tomorrow back, deciding when discernment is finished. Pray the six movements together, once, in silence, without sharing. Do not discuss members’ decisions from Lesson 18 in the group.

**Week Six: A Quieter Life (Lessons 19–21).** Peace before answers, the rule of life, the final prayer. Invite members to write a half-page rule in the week, not in the session, and to share one line if they wish. Close with the Final Prayer of Surrender, with the introductory line for uncertain readers read aloud.

**After the course.** Offer the 30-Day Mind-Renewal Journey for those who want it, and make clear it is optional. Follow up privately with anyone about whom you had concerns. Review the course with your co-leader and your safeguarding lead.

**A few practical boundaries.** Keep the group small enough to be cared for responsibly, normally no more than eight to ten, and establish passing as an ordinary option that requires no explanation. Start and end on time; a restless mind is not helped by a group that overruns. Do not add exercises of your own, and do not import material from deliverance ministry, inner-healing methods or therapeutic modalities you are not trained in. Do not use the sessions to identify who in the group “needs help”; if you are concerned about someone, speak with them privately. And do not run this course as a substitute for your church having a

proper referral relationship with local mental-health services; the course should make such referrals more likely, not less.

## One-to-One Use

Much of the above applies when a pastor or counsellor uses the course with one person. Some things are different.

Agree at the outset what the course is for in your work together, and what would lead you to suggest additional help. Revisit that agreement if the person's need turns out to be greater than a course can meet.

Let the person choose the pace and the chapters. The course is not a syllabus, and some chapters will not be relevant.

Be especially careful with Lessons 4, 7 and 18. Lesson 4's exercise ("If This Happened, What Would It Mean to Me?") can move quickly into material that belongs with a trauma-informed professional; if it does, stop, and do not press. Repair, Learn, Continue or Release must never be used to encourage contact with someone who has asked for none, or to bypass safeguarding or legal processes. The Faithful Decision Sheet must never be used to authorise an irreversible decision in a single conversation; the course requires appropriate review, and so should you.

If you are a counsellor working within a professional framework, that framework governs, and this course is a resource within it rather than a method.

And if you are a pastor, remember what the course says about yourself in Lesson 12: you are a person to whom people bring more than is yours to carry. Use the Support Map. Have your own rule. Know who you would call.

## **A Word About Theology in the Room**

The course has been careful, in every chapter, not to say that anxiety is weak faith, that healing follows sufficient prayer, that any biblical figure had a modern disorder, that Scripture's commands about anxiety condemn involuntary symptoms, or that any of its own tools are what the biblical author had in mind. It has marked its pastoral applications as such. It has said that Christians differ on the Sabbath, on signs and impressions, on the continuing application of some texts, and it has tried not to settle those differences on the reader's behalf.

Please hold the same line in your group. Members will bring the teaching they have received elsewhere, some of it harmful, and some of them will have been told that their restlessness is unbelief, that medication is a lack of trust, or that they would be well by now if they had enough faith. You do not need to attack those teachings. You do need to make sure they are not spoken as the group's teaching, and that the person who has been wounded by them hears something different from you. The course's central messages are not complicated: fear is an experience and trust is a direction; a prediction is not yet established; you may bring the same need to God as often as faithful prayer requires; Christ gave peace before the answers; you do not have to solve your whole life tonight. Say them, in your own words, until the room believes you.

## Leader's Checklist

Before the first session, I have:

- Read the whole course, including every help note and this section.
- Completed current safeguarding and suicide-awareness training, or ensured that a co-leader has completed it.
- Obtained and read my church's safeguarding and confidentiality policies, and identified the safeguarding lead.
- Identified where any required records will be stored.
- Established a safe way to contact every participant.
- If the group meets online, planned how to respond if someone becomes distressed and obtained each participant's physical location and emergency-contact procedure, as organisational policy permits.
- Written down current local referral routes and emergency numbers.
- Arranged a co-leader, or named someone outside the group whom I can consult.
- Identified someone with whom I can discuss my own responses to the material.
- Prepared an explanation of the group's confidentiality agreement, its limits and the fact that confidentiality cannot be guaranteed.
- Prepared a private alternative for every sharing activity.
- Decided which exercises will not be shared in the group.

During the course, I will:

- Never diagnose, score or categorise members.
- Never require disclosure.
- Never advise on medication.
- Never attempt mediation where abuse is disclosed.
- Respond to disclosures of suicide or self-harm as this section directs.
- Follow safeguarding procedures the same day when they apply.
- Refer promptly, kindly and practically, and continue appropriate pastoral care afterwards.
- Stop a member's use of a practice that has become compulsive, and help them find appropriate professional support.
- Preserve the course's theological boundaries in the group.

If I cannot tick every item, I am not ready to lead, and the right thing is to wait until I am.

The people who come to a group like this have often been carrying their minds alone for years. You will be, for six weeks, one of the places where they do not have to. That is a good thing to be. Be it within your limits, and let the limits be part of the care.

*Finding Help Where You Live*

# Group Agreement

*Printable. Read aloud at the first session, before any sharing, and give each member a copy.*

## What we promise each other

- **What is said in the group stays in the group.** We do not discuss one another's disclosures outside it, even with spouses.
- **Nobody has to share.** Passing is an ordinary option and requires no explanation. Every sharing activity has a private alternative.
- **Nobody's anxiety will be treated as weak faith.** Nobody will be diagnosed, scored, categorised or advised about medication.
- **We start and end on time.**

## Sharing or passing

When an exercise is discussed, you may share what you have written, share only the part you choose, say what the exercise was like without its content, or pass. "Pass" is a complete answer, and no one will ask why.

Two worksheets stay out of the group entirely: **"If This Happened, What Would It Mean to Me?"** and **The Faithful Decision Sheet**. You may say that you used them. You will not be asked what is on them.

## The limits of confidentiality

Your leaders will protect your privacy, and they will reinforce this agreement. They cannot guarantee the conduct of every participant outside the group, so share accordingly.

Information may need to be shared in these circumstances: a safeguarding concern, a serious risk of harm to you or to someone else, an applicable legal duty, or necessary consultation with a safeguarding lead or professional supervisor. In those circumstances your leaders will share only what is necessary, only with those who need to know, and will, wherever possible, tell you first what they intend to do.

Your leaders will not promise to keep secret anything that would leave someone in danger, and they will not make promises about what the police, social services or health services will do with information.

## What this group is not

This group is pastoral formation, not medical or psychological treatment. It does not treat anxiety disorders, depression, OCD or trauma. If a qualified professional is supporting you, their guidance takes priority over anything here, including the exercises and their limits.

If you need more than this group, that is not a failure of the group. Finding Help Where You Live shows how to find support in your country.

*This copy is for you to keep. It is not collected by Faithful Path. Some churches ask participants to sign and retain a copy under their own policy; if yours does, the signed copies must be stored in the organisation's approved secure system, not in a leader's personal notebook.*

Name (only if your church asks for it): ..... Date: .....

# Leader's Checklist

*Printable. Complete before the first session.*

Before the first session, I have:

- Read the whole course, including every help note and this section.
- Completed current safeguarding and suicide-awareness training, or ensured that a co-leader has completed it.
- Obtained and read my church's safeguarding and confidentiality policies, and identified the safeguarding lead.
- Identified where any required records will be stored.
- Established a safe way to contact every participant.
- If the group meets online, planned how to respond if someone becomes distressed and obtained each participant's physical location and emergency-contact procedure, as organisational policy permits.
- Written down current local referral routes and emergency numbers.
- Arranged a co-leader, or named someone outside the group whom I can consult.
- Identified someone with whom I can discuss my own responses to the material.
- Prepared an explanation of the group's confidentiality agreement, its limits and the fact that confidentiality cannot be guaranteed.
- Prepared a private alternative for every sharing activity.
- Decided which exercises will not be shared in the group.

During the course, I will:

- Never diagnose, score or categorise members.
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- Never advise on medication.
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- Respond to disclosures of suicide or self-harm as this section directs.
- Follow safeguarding procedures the same day when they apply.
- Refer promptly, kindly and practically, and continue appropriate pastoral care afterwards.
- Stop a member's use of a practice that has become compulsive, and help them find appropriate professional support.
- Preserve the course's theological boundaries in the group.

If I cannot tick every item, I am not ready to lead, and the right thing is to wait until I am.

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